

Name	: Mr. NITIN DESHMUKH	Age	: 32 Years
Lab No.	: 436622622	Gender	: Male
Ref By	: SELF	Reported	: 10/12/2023 9:17:04PM
Collected	: 10/12/2023 10:27:00AM	Report Status	: Interim
A/c Status	: P	Processed at	: LPL-NAGPUR LAB
Collected at	: AMAN QUWRESHI (MENGRE LAYOUT NAGPUR CC) c/o Satish Rajgire,, Santaji Co.op.so , Behnd khao galli,Mengre layout, Manish Nagpur		: Plot No. 7, Ground Floor Hotel Sunny International, Besides Gujrati Samaj Bhavan, Dhantoli Nagpur - 440012.

Test Report

Test Name	Results	Units	Bio. Ref. Interval
SwasthFit Super 4			
LIVER & KIDNEY PANEL, SERUM			
Creatinine	0.63	mg/dL	0.67 - 1.17
GFR Estimated	130	mL/min/1.73m2	>59
GFR Category	G1		
Urea	16.44	mg/dL	17.00 - 43.00
Urea Nitrogen Blood	7.68	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio	12		
Uric Acid	6.52	mg/dL	3.50 - 7.20
AST (SGOT)	34.9	U/L	<50
ALT (SGPT)	28.3	U/L	<50
GGTP	34.0	U/L	<55
Alkaline Phosphatase (ALP)	70.61	U/L	30 - 120
Bilirubin Total	0.36	mg/dL	0.30 - 1.20
Bilirubin Direct	0.07	mg/dL	<0.30
Bilirubin Indirect	0.29	mg/dL	<1.10
Total Protein	7.24	g/dL	6.40 - 8.30
Albumin	4.62	g/dL	3.50 - 5.20
A : G Ratio	1.76		0.90 - 2.00
Globulin(Calculated)	2.62	gm/dL	2.0 - 3.5
Calcium, Total	9.21	mg/dL	8.60 - 10.30



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Phosphorus	4.32	mg/dL	2.40 - 4.40
Sodium	135.40	mEq/L	136.00 - 146.00
Potassium	6.50	mEq/L	3.50 - 5.10
Chloride	101.70	mEq/L	101.00 - 109.00

Note

1. Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1

LIPID SCREEN, SERUM (CHO-POD)

Cholesterol, Total	159.04	mg/dL	<200.00
Triglycerides	211.95	mg/dL	<150.00
HDL Cholesterol	33.50	mg/dL	>40.00
LDL Cholesterol, Calculated	83.15	mg/dL	<100.00
VLDL Cholesterol, Calculated	42.39	mg/dL	<30.00
Non-HDL Cholesterol	126	mg/dL	<130

Note

1. Measurements in the same patient can show physiological & analytical variations. Three serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
2. Friedewald equation to calculate LDL cholesterol is most accurate when Triglyceride level is < 400 mg/dL. Measurement of Direct LDL cholesterol is recommended when Triglyceride level is > 400 mg/dL
3. Lipid Association of India (LAI) recommends screening of all adults above the age of 20 years for



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Test Name	Results	Units	Bio. Ref. Interval
	Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors		
4.	Indians tend to have higher triglyceride levels & Lower HDL cholesterol combined with small dense LDL particles, a pattern known as atherogenic dyslipidemia		
5.	Non HDL Cholesterol comprises the cholesterol carried by all atherogenic particles, including LDL, IDL, VLDL & VLDL remnants, Chylomicron remnants & Lp(a)		
6.	LAI recommends LDL cholesterol as primary target and Non HDL cholesterol as co-primary treatment target		
7.	Apolipoprotein B is an, secondary lipid target for treatment once LDL & Non HDL goals have been achieved		
8.	Additional testing for Apolipoprotein B, hsCRP, Lp(a) & LP-PLA2 should be considered among patients with moderate risk for ASCVD for risk refinement		

Treatment Goals as per Lipid Association of India 2020

RISK CATEGORY	TREATMENT GOAL		CONSIDER THERAPY	
	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHLOESTEROL (NON HDL-C) (mg/dL)
Extreme Risk Group Category A	<50 (Optional goal ≤30)	<80 (Optional goal ≤60)	≥50	≥80
Extreme Risk Group Category A	≤30	≤60	>30	>60
Very High	<50	<80	≥50	≥80
High	<70	<100	≥70	≥100
Moderate	<100	<130	≥100	≥130
Low	<100	<130	≥130*	≥160*

*In low risk patient, consider therapy after an initial non-pharmacological intervention for at least 3 months



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Test Name	Results	Units	Bio. Ref. Interval
COMPLETE BLOOD COUNT;CBC (Photometry, Electrical Impedance, Optical/Impedance & Calculated)			
Hemoglobin	14.62	g/dL	13.00 - 17.00
Packed Cell Volume (PCV)	42.90	%	40.00 - 50.00
RBC Count	5.18	mill/mm3	4.50 - 5.50
MCV	82.80	fL	83.00 - 101.00
MCH	28.20	pg	27.00 - 32.00
MCHC	34.10	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW)	14.20	%	11.60 - 14.00
Total Leukocyte Count (TLC)	7.05	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils	46.98	%	40.00 - 80.00
Lymphocytes	34.85	%	20.00 - 40.00
Monocytes	8.16	%	2.00 - 10.00
Eosinophils	9.63	%	1.00 - 6.00
Basophils	0.38	%	<2.00
Absolute Leucocyte Count			
Neutrophils	3.31	thou/mm3	2.00 - 7.00
Lymphocytes	2.46	thou/mm3	1.00 - 3.00
Monocytes	0.58	thou/mm3	0.20 - 1.00
Eosinophils	0.68	thou/mm3	0.02 - 0.50
Basophils	0.03	thou/mm3	0.02 - 0.10
Platelet Count	347	thou/mm3	150.00 - 410.00
Mean Platelet Volume	9.7	fL	6.5 - 12.0

Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
blood			
2. Test conducted on EDTA whole blood			



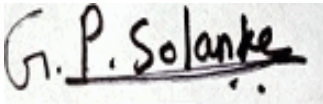
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Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE, FASTING (F), PLASMA (Hexokinase)	91.59	mg/dL	70.00 - 100.00
VITAMIN B12; CYANOCOBALAMIN, SERUM (ECLIA)	798.10	pg/mL	211.00 - 946.00

Notes

1. Interpretation of the result should be considered in relation to clinical circumstances.
2. It is recommended to consider supplementary testing with plasma Methylmalonic acid (MMA) or plasma homocysteine levels to determine biochemical cobalamin deficiency in presence of clinical suspicion of deficiency but indeterminate levels. Homocysteine levels are more sensitive but MMA is more specific
3. False increase in Vitamin B12 levels may be observed in patients with intrinsic factor blocking antibodies, MMA measurement should be considered in such patients
4. The concentration of Vitamin B12 obtained with different assay methods cannot be used interchangeably due to differences in assay methods and reagent specificity



Dr. Gaitri Solanke
MBBS,MD (Pathology)
Chief of Laboratory
Dr Lal PathLabs Ltd



Result/s to follow:

HbA1c (GLYCOSYLATED HEMOGLOBIN), BLOOD, THYROID PROFILE,TOTAL, SERUM, VITAMIN D, 25 - HYDROXY, SERUM



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IMPORTANT INSTRUCTIONS •Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory . •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician . •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner /Doctor. •The report does not need physical signature. (#) Sample drawn from outside source. If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action. Tel: +91-11-49885050, Fax: - +91-11-2788-2134, E-mail: lalpathlabs@lalpathlabs.com			

